



Abnormal and Clinical Psychology

HISTORIES OF ABNORMAL AND CLINICAL PSYCHOLOGY TEND TO BE decidedly presentist and generally represent the history of psychological theory and therapy as a progression from superstitious theories of spirit possession and brutal persecution to contemporary scientific theories and humane treatments (Sedgewick, 1982). Often this is based upon little more than condescending assumptions about earlier peoples. For example, when Neolithic skulls were discovered with holes in them, the French neurophysiologist Paul Broca opined that they must have been made “in order to liberate evil spirits” (Ackerknecht, 1982, pp. 8–9), a representation still repeated in contemporary histories of psychology. Yet it is just as likely that such holes were the product of early forms of trepanning, the removal of part of the skull to reduce swelling of the brain caused by injury through war or hunting.

As noted earlier, the ancient and medieval peoples were rather more sophisticated and humane, at least relative to their times, than they are usually given credit for. They recognized depression, mania, and hysteria and attributed most psychological disorders to neural causes. Their psychological treatments, which were usually based upon holistic principles derived from the Hippocratic school and Galen, included fresh air, relaxation, dieting, and music, as well as bloodletting and purgation.

From early Roman times, laws governed the treatment of the psychologically disturbed and provided for family or community guardianship of persons designated as “insane” or “mad” (Neaman, 1975). These laws recognized that such persons were not legally responsible for their actions, because of their diminished or defective powers of reasoning (Maher & Maher, 1985a; Neugebauer, 1978). Institutions devoted to the treatment of the psychologically disturbed were set up in medieval cities such as Baghdad, Valencia, and London, and the common law of many medieval European states included protections for them (Schoeneman, 1977).

Those deemed insane were generally cared for by their families or legally appointed guardians. They were occasionally beaten—the common law of England allowed people to beat their mad relatives (Allderidge, 1979)—and those deemed dangerous were restrained or imprisoned. Although no doubt many were persecuted

and exploited, their treatment was not especially cruel relative to the conditions of the time. Michael McDonald's (1981) analysis of the notebooks of Richard Napier (1559–1634), a 17th-century English physician, indicates that less than 1 percent of those who consulted him for psychological problems complained of beatings or other forms of abuse.

Eventually the insane became the legal responsibility of local parishes and urban centers, which were also responsible for the poor and the unemployed. They were housed in asylums, originally created to isolate lepers and later employed to house a wide variety of “degenerates” and “destitutes.” This was as much a social as a medical measure, designed to combat the growing problems of begging and vagrancy, as well as occasional acts of violence. For example, the departments of the Hôpital Général of Paris, La Bicêtre for men and Salpêtrière for women, were created to confine beggars, tramps, and prostitutes along with the insane and the immature, by order of a royal edict of 1656 (Rosen, 1968).

While many institutions were originally intended as humane retreats, they evolved into places of incarceration. St. Mary's of Bethlehem in London, founded in 1247, degenerated into the human zoo that came to be known as “Old Bedlam” (after a corruption of the Cockney pronunciation), where inmates were chained



Ramble Gripe restrained by orderlies in Bedlam (Hogarth).

and whipped, regularly “treated” with purgatives, emetics, and bloodletting, and subjected to the indignity of public display before paying visitors. Yet although conditions in places like Bedlam were undoubtedly grim, they were probably not much worse than the working and living conditions of most ordinary people at the time. Although occasionally used as a form of treatment, restraints were generally employed only with violent patients. Those with manageable disorders were allowed out to beg as long as they wore identifying badges, and they became known as “Tom o’ Bedlams.” The original intent of charging visitors to view the inmates was to raise money to support their treatment.

NEUROSES, ALIENISTS, AND PSYCHIATRY

Robert Burton (1577–1640) was one of the first physicians to develop a systematic classification of psychological disorders and their causes and treatment in *Anatomy of Melancholy* (1621). Although his compilation was based upon Galen’s theory of humors and animal spirits, he distinguished mania from melancholy and documented a wide variety of causes of melancholy, such as poor child rearing and education, excessive love, religious fervor, bereavement, poverty, isolation, and old age (Millon, 2004). A chronic depressive himself, Burton believed that it was difficult but not impossible to relieve melancholy through treatment.

William Cullen (1710–1790), professor of medicine at the University of Edinburgh, who produced a comprehensive classification of physical and psychological disorders in *First Lines of the Practice of Physick* (1777), characterized as **neuroses** those physical and psychological disorders caused by damage to or disease of the nervous system. His colleague Robert Whytt described the form of nervous exhaustion later characterized as **neurasthenia** by the American neurologist George Beard (1839–1883). The Swiss physician Felix Plater (1536–1614) used the term **mental alienation** to describe the troubled condition of those suffering from psychological disorders, and up until the early 20th century physicians engaged in their diagnosis were characterized as **alienists** (the term is sometimes still used to characterize forensic psychiatrists). Johann Christian Reil (1759–1813), professor of medicine at the University of Halle, introduced the term **psychiatry** to describe the study and treatment of psychological disorders. Johann Christian Heinroth (1773–1843) was appointed to the first chair in psychiatry at the University of Leipzig in 1811 and revived the ancient term **paranoia** to describe a variety of disorders based upon intellectual and emotional disturbance.

In *Observations on Madness and Melancholy* (1809), the English physician John Haslam (1764–1844) provided detailed descriptions of the phases of manic depression (bipolar disorder) and the symptoms of the degenerative disorder that the Belgian psychiatrist Bénédict-Augustin Morel (1809–1873) later classified as **dementia praecox**, because of its early onset and rapid progression (the disorder

had also been recognized by the Roman physician Aretaeus in the first century and by Thomas Willis in the 17th). Morel came to believe that degeneration is characteristic of all psychological disorders and followed the French physician Jacques-Joseph Moreau (1804–1884) in claiming that mental degeneration is determined by heredity and impervious to treatment.

Social Darwinists readily embraced this view and endorsed Morel's claim that degeneracy constituted the "greatest obstacle" to social progress (Morel, 1857). The German psychiatrist Richard von Krafft-Ebing (1840–1902), a follower of Morel's, claimed that hereditary degeneracy was the cause of criminality, sexual perversion, and social unrest among the lower classes (Krafft-Ebing, 1879). Similar views were advanced by Cesare Lombroso (1835–1909), the Italian professor of psychiatry and criminal anthropology, who suggested that degeneracy could be identified via abnormalities in the shape of the head, eyes, ears, and jaw (Lombroso, 1876), and by Felix Voisin (1794–1872), a disciple of the phrenologists Gall and Spurzheim.

Not all theorists held that psychological disorders were a product of inherited degeneracy or that they were especially characteristic of the lower classes. Daniel Noble identified chemical causes of psychological disorder, based upon his own empirical explorations of the effects of alcohol, cannabis, and opium, whose use was by no means limited to the lower classes (Noble, 1853). William Moseley recognized a variety of social causes (Moseley, 1838) and attributed the high rates of psychological disorder in middle- and upper-class women in Victorian society to the lack of exercise of their mental faculties (Maher & Maher, 1985b).

The British physician James Cowles Prichard (1786–1848) was one of the first to adopt an explicitly moral attitude to psychological disorder. He treated "moral insanity" as a defect of character, the result of allowing oneself to be swayed by "affections" rather than by "natural feelings" of rightness and responsibility. Consequently many early-19th-century treatments of psychological disorder were conceived of as forms of **moral management**, directed to the transformation of character through personal counseling and religious training.

THE REFORM OF ASYLUMS

Among those who conceived of the treatment of psychological disorder as a form of moral management was William Battie (1704–1776), the founder of St. Luke's Hospital in London. He distinguished between "original" (inherited) and "consequential" forms of madness in his *Treatise on Madness* (1758). Battie claimed that the latter could be treated by isolation from the influence of family and friends, which he held to be the social cause of madness. Many attributed psychological disorders to the unhealthy conditions of cities, for which reason melancholy was often treated as the English malady (Cheyne, 1734). Consequently many asylums were located in the country and conceived of as retreats from the traumatic conditions of city life.

William Tuke (1732–1822), a retired English tea and coffee merchant, set up a retreat in York as a refuge for psychologically disturbed Quakers in 1796, based upon principles of moral management. He treated inmates at the York retreat with respect and dignity, in a rural setting designed to resemble a farmstead. Tuke developed a treatment regime of exercise and wholesome diet, supplemented by pastoral counseling and occupational therapy. He eliminated barred cells and physical restraints (except in violent cases) and prohibited bloodletting and beating. His son and his grandson Daniel Hack Tuke (1827–1895) continued the York retreat. Daniel traveled to the United States and promoted the humane treatment of the insane through meetings with the superintendents of American asylums, which he considered to be superior to most British and European institutions (Millon, 2004). The York retreat stimulated the creation of similar retreats in Europe and the United States, such as the Friends Asylum for the Use of Persons Deprived of the Use of Their Reason, founded in Philadelphia in 1813.

Juan Luis Vives and Johann Weyer had advocated the humane treatment of the insane in the 16th century (Reisman, 1991), but their noble ideals had been neglected as asylums had degenerated into places of incarceration for the destitute and degenerate as well as those suffering from genuine psychological disorders. By the end of the 18th century, public concern about the conditions in such institutions inspired various reform movements, which resulted in government commissions and regulations governing the treatment of the insane. A British Act of 1774 mandated medical supervision and restricted occupancy to those certified as insane, to exclude the dumping of troublesome relatives. In France the Code Napoléon was modified in 1838 to include regulations governing the certification of the insane and the establishment and inspection of public asylums. Such reforms were more a product of Enlightenment ideals than scientific developments in theory and treatment, although the pioneers of reform did introduce empirical methods of classification and evaluation and emphasized the need for accurate statistical records of the efficacy of treatments (Earle, 1838/1887; Pinel, 1801/1806).

Vincenzo Chiarugi (1759–1820), superintendent of the Ospedale di Bonifazio in Florence, initiated a number of humanitarian reforms, including the limitation of physical restraints on patients. He eliminated shackles and chains and permitted restraint by straitjacket only for the violent and delirious. In *On Insanity and Its Classification*, he avowed that physicians are obliged to “respect the insane individual as a person” (1793/1987, p. 24). The French physician Joseph Daquin (1732–1815) recommended a similar humanitarian approach to the treatment of the mentally ill at the French hospital in Chambery, including the removal of shackles and other forms of physical restraint (Millon, 2004). Like Chiarugi, he recommended that the insane should be treated in isolation from the broader community and those incarcerated for different reasons, such as the poor and the degenerate (Daquin, 1791).

These early contributions were eclipsed by the work of Philippe Pinel (1745–1826), who was appointed head of the Bicêtre Hospital in Paris during the French Revolution. From 1793 to 1795 Pinel initiated a variety of reforms, which included the removal of chains from inmates and the elimination of practices such as blood-letting, beating, and confinement. Although Pinel usually receives the credit, it was Jean-Baptiste Pussin who initiated the practice of removing restraints. Pussin was a former inmate of the Bicêtre who was put in charge of incurable cases (Weiner, 1979).

Pinel continued these reforms when he was appointed director of the Salpêtrière hospital in 1795. This was the Paris asylum for insane women, with around 8,000 inmates. Pinel and Pussin oversaw the removal of chains from inmates and instituted the same humane treatment regimes (which included forms of occupational therapy) that had produced impressive improvement rates at the Bicêtre. Pinel developed a classification of psychological disorders based upon case studies of inmates and their treatment progress (1798, 1801). He instituted a system that assigned inmates to separate wards according to their diagnostic types, which was duplicated by many of his disciples, but criticized by the Belgian alienist Joseph Guislain (1797–1860), who complained that grouping the depressive, the apathetic, and the violent together only exacerbated their condition (Millon, 2004).

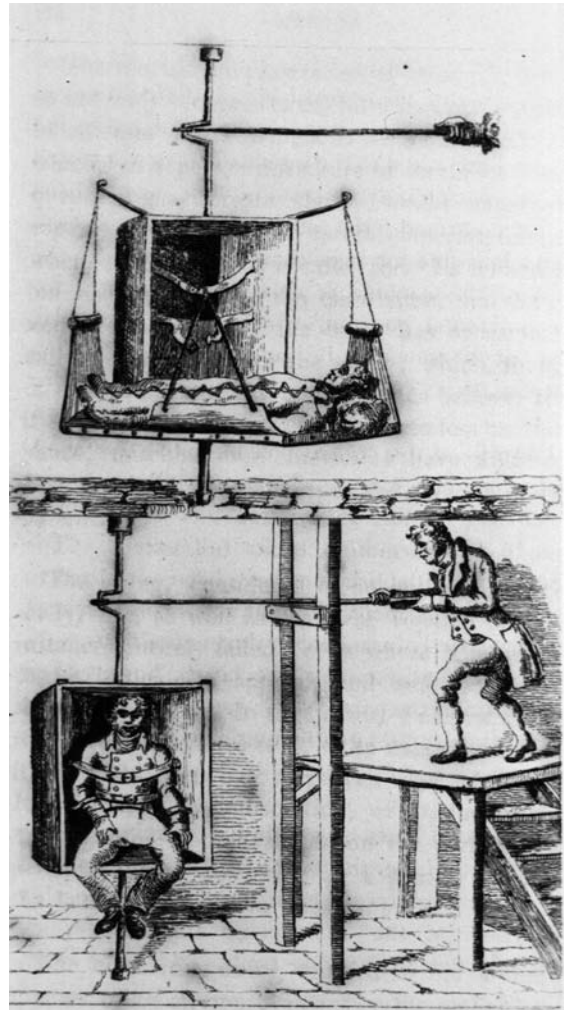
Jean-Étienne-Dominique Esquirol (1772–1840), Pinel's disciple and successor at the Salpêtrière, tried to promote his humane treatment regime throughout Europe, but with limited success. John Connolly (1784–1860) and Joseph Guislain (1797–1860) initiated similar reforms in England and Belgium, but progress remained slow, given the sustained opposition by most physicians. In the United States, pioneers such as Benjamin Rush (1745–1813) and Eli Todd (1762–1832) championed the humane treatment of the mentally ill and the reform of asylums. Rush, who had trained with Cullen in Edinburgh, deplored the cruelty and custodial nature of the institutional treatment of the insane in *Medical Inquiries and Observations Upon the Diseases of the Mind* (1812). He became superintendent of the Pennsylvania Hospital, where he petitioned the governors for better facilities for the treatment of inmates. Yet Rush, who is often treated as the father of American psychiatry, retained many doubtful practices. He remained an enthusiastic supporter of bloodletting and promoted the use of restraining and “spinning” chairs, later known as “Rush” chairs, which were supposed to calm patients through induced dizziness.

Eli Todd founded the Connecticut (or Hartford) Retreat for the Insane in 1824 with money raised by public subscription. He introduced a program of moral management based upon the pastoral counseling provided at the York Retreat, which became a model for the type of care provided at a number of institutions founded in the early 19th century. Samuel Woodward (1787–1850), a disciple of Todd's, promoted the moral management of the insane while superintendent of Worcester State Hospital in Massachusetts, as did Pliny Earle (1809–1892), the

superintendent of Bloomington Hospital in New York. Earle championed the use of detailed statistical records of psychological treatment to counter what he believed to be overinflated treatment success rates claimed by his colleagues (Earle, 1848) and published an account of the principles of moral management (Earle, 1838/1887). He characterized it as a form of treatment designed to restore the “mental faculties” to their optimal level of functioning through rest, relaxation, and the cultivation of reason and self-control.

However, the sheer numbers of inmates in public asylums, which were inundated with the poor and the criminal, forced many superintendents to abandon moral management and resort to beatings, cold baths, and physical restraint. Appalled by the conditions that had developed in American asylums, Dorothea Dix (1802–1887) devoted her life to their reform. Dix originally worked as a teacher at a Boston prison for women and was moved to press for reform when she discovered that women suffering from psychological disorders were incarcerated along with common criminals. She traveled from state to state for about 40 years, and harnessed the press and public in her effort to persuade legislatures to reform or create new institutions for the insane (Reisman, 1991).

A number of Dix’s disciples instituted her reforms while serving as superintendents at major state institutions: these included John S. Butler (1803–1878) at Boston Lunatic Hospital and Worcester State Hospital and John M. Galt (1819–1862) at the Williamsburg Asylum, the first public institution for the care of the insane, founded in 1773 (Zwelling, 1985). Dix gained her reputation as an American Florence Nightingale when she served as chief of hospital nurses during the American Civil War, and she exploited that reputation when she later toured Europe to promote her cause. She lectured Queen Victoria and Pope Pius IX on the need for reform of the institutional treatment of the insane, with some degree of success. Queen Victoria appointed a royal commission to investigate the treatment of the insane in Britain, and the pope established a new asylum in Rome (Reisman, 1991).



Rush “spinning chair.”

However, her achievements were modest and limited in her own country. American Social Darwinists questioned the wisdom of supporting constitutional mental degenerates, and Dix failed in her attempt to have Congress institute a land-grant bill to finance the establishment of psychiatric hospitals in the way that the Morrill Act supported the development of state universities. Many institutions continued with traditional forms of incarceration and physical restraint, along with “treatments” such as bloodletting, purging, spinning, electrical stimulation, and the ingestion of chemicals such as strychnine and mercury.

MAGNETISM, MESMERISM, AND HYPNOSIS

Although theories of physical and psychological health and disease based upon bodily humors fell out of favor in the 17th and 18th centuries, theories based upon the balance of physical forces remained popular. Luigi Galvani and Alessandro Volta claimed that electricity was the basis of life and mind, a view that gained increasing acceptance with the 19th-century investigation of the electrical nature of neural transmission. Such views formed the basis of Erasmus Darwin’s theory of “fluid materialism” and Mary Shelley’s *Frankenstein*.

Electrical treatments of physical and psychological disorders became popular, especially in the latter part of the 19th century. The employment of electrical stimulation as a medical treatment goes back to ancient times (the Roman essayist Pliny recorded how electric eels were used to relieve the pain of childbirth) but accelerated in the 19th century through technological advances in the measurement and application of electric current. Electropathic belts and direct electrical stimulation of the body and brain were touted as cures for ailments ranging from headaches to neurasthenia and were employed by physicians such as James and Freud. In the late 19th century John Birch reported improvement after he applied an electric current to the head of a depressed patient at St. Thomas’s Hospital in London (Clare, 1976, cited in Maher & Maher, 1985b), a technique developed in the 20th century as electroconvulsive treatment (ECT).

However, at least as influential was the view that magnetic force forms the basis of life and mind. Gilbert had conceived of magnetism as a universal force, and Newton had suggested that gravitation is a form of magnetism. This led to the idea that magnetism is a cosmic force that governs every aspect of the universe, including the body and mind. Physical and psychological health came to be treated as a function of the proper alignment of magnetic forces in the body and brain, and disease and disorder as a product of misalignment. This naturally led to the view that cures of physical and psychological disorders could be affected by the application of magnetic force through the use of physical magnets, a view promoted in the 18th century by Franz Anton Mesmer (1734–1815).

Mesmer gained a medical degree from the University of Vienna in 1766 with a dissertation on planetary influences. He claimed to have identified the invisible “subtle and mobile fluid” that is the medium of magnetic force and that pervades every aspect of the universe. He supposed that the planets influence the distribution of magnetic fluids in the body and that “animal magnetism” is a product of “animal gravitation”:

Those spheres . . . exert a direct action on all the parts that go to make up animate bodies, in particular on the nervous system, by an all-penetrating fluid.

—(1779/1948, p. 3)

Mesmer claimed that physical and psychological disorders are caused by obstructions to the free flow of magnetic fluid in the body and brain and that relief can be attained only through the restoration of magnetic balance. He originally employed steel magnets and other physical media such as magnetized water in his “magnetic” treatments, but eventually claimed that some individuals (including himself) had such strong personal magnetic fields that they were able to manipulate the fields of others by merely moving their hands in the proximity of their bodies.

Mesmer was ridiculed as a charlatan in his native Vienna and condemned by physicians, scientists, and the clergy, who thought him in league with the devil. However, his theory and practice were more enthusiastically received in Paris, where he set up a salon in 1788 and shifted from individual to more lucrative group treatments. These involved elaborate apparatus and ceremonies. To the accompaniment of music, attendants used magnetic rods to connect patients to a bath containing “magnetized water” and instructed them to rub their bodies with the ends of the rods. After an hour or so of this magnetic experience, Mesmer would appear in an elaborate purple silk robe and wave his magnetized wand over them. He claimed great success for this form of treatment, which came to be known as **mesmerism**. It attracted the enthusiastic support of Marie Antoinette, the French queen, and the marquis de Lafayette, the American revolutionary hero.

Mesmer’s success in relieving physical and psychological disorders was almost certainly a product of suggestion—he regularly advised patients of the anticipated outcome of their treatments. This was the conclusion of the royal commission that Louis XVI established in 1784 to evaluate the efficacy of mesmerism as a medical treatment. Benjamin Franklin (1706–1790) chaired the commission, whose members were drawn from the French Academy of Sciences. They included the distinguished French scientist Antoine Lavoisier (1743–1794) and Dr. Joseph Ignace Guillotin (1738–1814), who did not invent the instrument of execution that bears his name, but merely recommended it as a more humane form of execution. When the commission published its report in 1784,



Mesmerist represented as manipulating the magnetic field of a subject.

it concluded that Mesmer was a fraud and attributed the positive effects of magnetic treatments to the imagination of patients:

Imagination apart from magnetism produces convulsions, and . . . magnetism without imagination produces nothing.

—(Beloff, 1975, p. 268)

The report effectively destroyed Mesmer's career, and he left Paris the following year. However, despite vigorous opposition from the medical and religious establishment, many mesmerists remained committed to the view that their form of treatment was grounded in the manipulation of invisible magnetic fluids. As late as the end of the 19th century, Albert Binet, the pioneer of intelligence testing in France, claimed to have demonstrated the transfer and polarization of sensations via the use of magnets (Binet & Féré, 1887). This almost destroyed Binet's own

career, when other French researchers demonstrated that similar effects could be attained through mere suggestion.

Others continued to employ mesmeric treatments but abandoned the theory of magnetic fluids. Mesmer had managed to induce a state of trance in patients by merely commanding them to sleep, by staring intently at them and exclaiming “Dormez!” The French nobleman Armand M-J de Chastenet (1751–1825), the marquis de Puységur, rejected the theory of magnetic fluids, but became a successful practitioner of mesmeric techniques. He founded the Society of Harmony in 1785 to promote mesmerism, and his reputation spread throughout Europe, much to Mesmer’s displeasure. Puységur discovered that he could induce temporary states of paralysis in patients during periods of “mesmeric” or “artificial” somnambulism: They could be commanded to speak, dance, and perform mechanical tasks, with no recollection of their actions upon awakening. He demonstrated the therapeutic effects of what he called “nervous sleep,” including its anesthetic properties: He was able to raise a patient’s threshold of pain while in a trance state.

John Elliotson (1791–1868), professor of medicine at University College, London, suggested that such artificially induced trance states could be employed as a surgical anesthetic. He performed a number of operations using the technique, including some amputations (Gravitz, 1988), as did James Esdaile (1808–1859), an East India Company surgeon stationed in Calcutta (Esdaile, 1846, 1852). Yet the medical establishment remained skeptical and critical. The Governing Council of University College admonished Elliotson in 1837 with a resolution banning the use of mesmeric techniques, and he promptly resigned his position. When Esdaile returned home to Scotland and tried to employ mesmeric techniques at Perth Royal Infirmary, the medical authorities proscribed his work (Beloff, 1975). Their early successes were soon forgotten with the development of chemical anesthetic agents such as ether and chloroform, which seemed more reliably effective than induced trance states, and the surgical use of mesmeric techniques was abandoned.

Elliotson later explored the use of mesmeric techniques as forms of treatment for psychological disorders and founded the journal *The Zoist: A Journal of Cerebral Physiology and Mesmerism and Their Application to Human Welfare* in 1843 (Beloff, 1975). He claimed that mesmeric techniques were the most effective treatment for hysterical disorders, such as forms of paralysis or blindness that have no physiological basis, later characterized as “conversion hysterias” (Matarazzo, 1985). The Scottish surgeon James Braid (1795–1860) explained these treatment effects in terms of suggestibility and the operation of unconscious psychological processes. Braid, who had attended a demonstration of mesmeric techniques by a traveling French mesmerist, was originally skeptical, but became convinced of their efficacy when he found that he could induce trance states in members of his own family and command them to perform tasks of which they were consequently amnesic. In *Neurhypnology, or The Rationale of Nervous Sleep, Considered in Relation With Animal Magnetism* (1843), he characterized the trance state as

a form of artificial sleep and treated mesmeric techniques as forms of suggestion operating on the unconscious mind. Braid introduced the term **hypnosis**, from the Greek *hypnos*, meaning “sleep,” to describe the state of “nervous sleep” induced by mesmeric treatments (by dropping the “neur” from “neurohypnology”). Like Puységur, Braid repudiated the theory of magnetic fluids and claimed that he had “entirely separated Hypnotism from Animal Magnetism” (Braid, 1843, p. 112). His exploration of hypnotic treatment effects lent a degree of respectability to their study, but it was many years before the medical establishment took them seriously.

Jean-Martin Charcot (1835–1893), director of the Salpêtrière hospital, made important contributions to the study of multiple sclerosis and “shaking palsy” (Parkinsonism), and he became famous for his weekly public lectures, which attracted physicians from all over Europe and America, including Freud and James. In later years he turned his attention to the investigation of hysteria. Through the use of hypnosis, he dramatically induced hysterical symptoms such as anesthesia, paralysis, and falling fits in his female patients (although there is some suspicion that many of these were staged [Maher & Maher, 2003]).

Charcot believed that hysteria is a form of mental pathology based upon congenital degeneracy of the nervous system and that those subject to such degeneracy are susceptible to hypnotism. Since he claimed that hysteria and susceptibility to hypnosis are products of the same underlying neurological deficiency, he



Charcot's demonstration of female hysteria at the Salpêtrière.

maintained that susceptibility to hypnosis is a symptom of hysteria and that only hysterical patients can be hypnotized. Although he was one of the first to recognize cases of male hysteria, Charcot thought that hysteria is many times more common in women and believed that female hysteria is related to ovarian compression. (Hippocrates had earlier traced it to the uterus.)

Ambroise-Auguste Liébauld (1823–1904), one of the physicians who repudiated Binet's claims about the efficacy of magnetic treatments, challenged Charcot's account of hysteria. Liébauld practiced in a rural community near Nancy in France and began to use hypnosis as a therapeutic technique after he heard of Bain's work. He rejected Charcot's treatment of susceptibility to hypnosis as a symptom of hysteria and his theory that both are caused by degeneracy. In *Sleep and Analogous States* (1866), Liébauld reported that a good many hysterical patients are resistant to hypnosis and that some perfectly normal, stable, and well-balanced persons are extremely susceptible. Hippolyte Bernheim (1840–1919), professor of medicine at the Nancy School of Medicine, repeated Liébauld's challenge. Bernheim was impressed with Liébauld's therapeutic achievements and came to employ hypnotic techniques in his own clinic.

Liébauld and Bernheim held that suggestion is the primary vehicle of hypnotism and its therapeutic effects. They claimed that suggestibility is a normal and universal psychological trait and that every person can be hypnotized—although some are more suggestible and easier to hypnotize than others. Bernheim developed this central thesis of what became known as the **Nancy School** in *Hypnosis and Suggestion in Psychotherapy* (1865), which eventually prevailed in medical circles and which Charcot endorsed in later years. Liébauld and Bernheim employed hypnosis extensively in their treatment of hysteria and other psychological disorders and found that later positive suggestions could eliminate the harmful effects of earlier negative or "pathological" suggestions.

This latter insight was developed by Pierre Janet (1859–1947), one of Charcot's students, who became director of the neurophysiological clinic at the Salpêtrière. Janet claimed that certain painful thoughts, feelings, and desires become dissociated from normal consciousness and form separate psychological systems, or dissociated aspects of personality. Although these are normally unconscious, they are manifested in hysterical symptoms such as conversion paralyses—forms of physical paralysis, blindness, or analgesia that do not have discernible physical or physiological bases. Janet speculated that hysterical symptoms are a product of unconscious memories, which have their origin in earlier traumatic incidents. He investigated the medical records of patients at the Salpêtrière and noted that in many cases some physical shock or psychological trauma preceded the onset of hysterical symptoms. He suggested that hysterics have split personalities, one of which operates unconsciously as a secondary personality that can "invade" the primary personality (Janet, 1889, 1893). Janet claimed that this secondary personality could be revealed through hypnosis, which he used to explore

patient memories of past traumatic incidents. He discovered that the “psychological analysis” of such memories, during which patients expressed the emotion associated with the trauma, frequently brought relief of their hysterical symptoms—an insight famously developed by Freud (Ellenberger, 1970). In later years Janet claimed that Freud had plagiarized his ideas, and he became a vocal critic of Freud’s theory of the sexual etiology of most hysterical symptoms.

FREUD AND PSYCHOANALYSIS

The individual who had the greatest impact on the developing fields of psychiatry and abnormal psychology in the early decades of the 20th century was Sigmund Freud (1856–1939). He was born in the town of Freiberg in Moravia, a province of the Austro-Hungarian Empire, on May 6, 1856, the third of 8 children. Shortly afterward, the family moved to Vienna, where Freud lived until 1938, when he was forced by the Nazis to flee to London. He excelled at school and had a special talent for languages. When the University of Vienna withdrew its policy of excluding Jewish students in the 1860s, Freud seized the opportunity. His reading of Darwin’s *Origin of Species* reputedly stimulated his interest in science, and he joined the medical school in 1873.

During his training, Freud took elective courses in psychology with Franz Brentano and became a research assistant to Ernst W. von Brücke, one of the founding members of the Berlin Physical Society. As a student Freud embraced the reductive physicalism of Brücke, du Bois-Reymond, and Helmholtz, although he later abandoned it when he developed his psychodynamic theories. After he received his medical degree in 1881, he worked with the neuroanatomist Theodor Meynert (1833–1892) on the diagnosis of brain damage.

In 1884 Freud began to experiment with the use of cocaine and became an early and enthusiastic advocate of its analgesic and therapeutic properties. He supplied it to his relatives, friends, and fiancée; prescribed it to his patients; and published half a dozen papers lauding its beneficial effects. He was severely criticized by his medical colleagues when the dangers of cocaine addiction became increasingly clear. Freud’s incautious celebration of cocaine as a therapeutic agent in the absence of proper empirical evaluation was one reason why many in the medical community were skeptical when he first developed his psychodynamic theories. He managed to avoid the cocaine addiction to which many of his friends and patients succumbed, but never surmounted his addiction to nicotine. He smoked around 20 cigars a day, even after he was diagnosed with cancer of the palate and jaw. He later endured a series of 33 operations during which his jaw was replaced by an artificial device that he called “the monster.”

Freud originally planned to continue his work on brain damage, but his research interests changed when he visited Charcot’s clinic in the winter of 1885.

He was greatly impressed by Charcot's public demonstrations of female hysteria and his ability to induce and relieve hysterical symptoms through hypnosis. He was also intrigued by Janet's suggestion that hysteria is the product of repressed memories of traumatic events isolated in an unconscious secondary personality. Freud translated Charcot's lectures and became interested in conversion hysteria, which he recognized could not be accommodated by the form of reductive neurophysiological analysis that he had employed in his years of research with Brücke and Meynert (Bolles, 1993).

Studies on Hysteria

On returning to Vienna, Freud married Martha Bernays and settled down to private practice. He specialized in nervous diseases and employed fashionable electrical and water treatments, as well as explicitly directive and suggestive treatments. His friend Joseph Breuer (1842–1925), an associate of Meynert's whom Freud had met while a research student, referred many of his early patients. Although Freud usually claimed sole credit for the development of psychoanalytic theory and repudiated the suggestion that Janet had anticipated its basic principles, he credited Breuer with the discovery of the psychoanalytic method, through his treatment of a patient known as Anna O.

Anna O had developed a number of hysterical symptoms while nursing her dying father. These included visual impairment, paralysis of her arms and legs, speech disorientation, and refusal to drink water from a glass. During one of the periodic trances into which she frequently lapsed, Anna acted out her emotional response to an episode that had occurred some months earlier, during which she had been disgusted by the sight of a dirty dog drinking water from a glass. When she recovered from the trance, Breuer found that she was now able to drink. He was able to relate her hysterical symptoms to earlier emotional traumas, most of which related to her fear and guilt concerning the impending death of her father. He also found that he could relieve Anna's symptoms by having her discharge her negative emotions through reenactment of the original traumatic episode.

Breuer called this therapeutic method the "talking cure," after Anna's own description, and Freud began to employ a similar method with his own patients. He and Breuer co-authored a paper "On the Psychological Mechanism of Hysterical Phenomena: Preliminary Communication" in 1893, and published *Studies on Hysteria* in 1895. In this latter work they articulated the main theoretical components of psychoanalysis. They claimed that hysterical symptoms are expressions of repressed memories of earlier (often childhood) traumas, which can be relieved through the cathartic expression or **abreaction** of these emotionally charged memories, a process that Anna O called "chimney sweeping." They identified the clinical phenomenon of **transference**, in which feelings originally directed

to parents are redirected to the therapist, and **countertransference**, in which the therapist develops an emotional attachment to the patient. The marital tensions generated by these emotional attachments caused Breuer to discontinue Anna's treatment and his general use of the "talking cure."

Still in the grip of Brücke's reductionist approach, Freud completed *Project for a Scientific Psychology* in 1895 (although the work was only published posthumously in 1950), in which he tried to provide a neurophysiological account of the basic psychological processes presupposed by psychoanalytic theory (for example, he treated the emotional charge of a pathological idea as a quantifiable degree of neuronal excitation). He continued to employ the "talking cure" in his treatment of hysterical patients and began to use hypnosis after his visit to Bernheim's Nancy clinic in 1889. Freud was impressed by Bernheim's ability to relieve hysterical symptoms through hypnosis and by his demonstration of post-hypnotic suggestion, which illustrated the power of unconscious ideas to influence behavior. However, Freud abandoned hypnosis when he found he could identify pathogenic ideas by simply having patients relax and describe whatever came into their mind, no matter how apparently trivial or potentially embarrassing it might be—a method he called **free association**. He came to believe that the essential component of psychoanalytic treatment is the cathartic expression of emotions associated with repressed memories, however they may be identified, through spontaneous trance states, hypnosis, free association, or, as he later maintained, through dreams, jokes, and slips of the tongue.

In *Studies on Hysteria* (1895/1953) and "The Etiology of Hysteria," a paper presented to the Psychiatric and Neurological Society in Vienna in 1896, Freud claimed that the repressed memories responsible for neurotic symptoms are invariably memories of childhood sexual abuse—a claim rejected by Breuer, who maintained in a separate conclusion to *Studies on Hysteria* that repressed memories are not necessarily sexual. However, Freud soon abandoned this **seduction theory**. He came to believe that hysterical symptoms are **wish fulfillments**, symbolic expressions of repressed memories and desires that have their source in childhood sexual fantasies. Freud later maintained that this fundamental change in theoretical orientation marked the real beginning of psychoanalysis.

Psychosexual Development

Freud began to employ dream analysis in conjunction with free association in his therapeutic practice, since he came to believe that dreams are also symbolic wish fulfillments. This was a product of his own self-analysis, begun in 1896 to alleviate his extreme depression over the death of his father. In *The Interpretation of Dreams* (1900), Freud claimed that psychoanalysis is able to uncover the repressed **latent content** of dreams, as opposed to their reported apparent or **manifest**

content. Since he believed that the repression of traumatic memories is weaker during dreaming than waking consciousness, Freud characterized dreams as “the royal road to the unconscious.” *The Interpretation of Dreams* was followed by the *Psychopathology of Everyday Life* (1901), in which Freud argued that the apparent accidents and errors of everyday life are also symbolic wish fulfillments, now generally known as **Freudian slips**. Freud extended this analysis to jokes in *Jokes and Their Relation to the Unconscious* (Freud, 1905/1966).

One of the avowed discoveries of Freud’s self-analysis was the **Oedipus complex**. In his treatment of a recurrent childhood dream, Freud identified his hostility toward his father and desire for his mother and explained his overreaction to his father’s death as due to guilt about an earlier death wish relating to his father. He made the Oedipus complex the cornerstone of his theory of psychosexual development. According to Freud, sexual satisfaction is associated with different erogenous zones in the course of child development, during the so-called oral, anal, phallic, latency, and genital stages. He held that too much or too little stimulation at any of these stages leads to fixation of the relevant needs, leading to the development of personality types associated with these stages, such as the celebrated oral and anal personalities.

According to Freud, the most important developmental stage for understanding both normal and abnormal adult behavior is the phallic stage, during which the Oedipal conflict is generated. Between the ages of 3 and 6, male children develop strong sexual desire for their mother and come to treat their father as a rival. The active erogenous zone during this period is the genital area, and the male child experiences castration anxiety, based upon fear of the more powerful father. The Oedipal conflict is resolved through the child’s identification with the father, which gains him symbolic access to his mother and removes the fear of castration, although only temporarily, since repressed desires continue to influence adult dreams and behavior. Freud maintained that female children experience a similar but qualitatively distinct form of conflict around the same time, which he originally called the “Electra complex,” although he later abandoned the term. They also develop a castration complex, which is resolved in an analogous fashion through identification with the mother.

This process of identification with the parent of the opposite sex was the supposed vehicle for the development of what Freud called the **super-ego**, through the internalization of the moral principles of the parent of the opposite sex. In his theory of personality, Freud distinguished between the id, the ego, and the super-ego. The **id** represents the instinctual engine of human psychology, comprising basic drives and desires, such as hunger, thirst, and sexual desire, which underlie all behavior. The id operates on the **pleasure principle** and strives to attain the gratification of basic drives and desires, which is by nature pleasurable. One of the primitive instinctual mechanisms is symbolic wish fulfillment, in which an image representing an object satisfying a desire is generated. However, basic drives and

desires are more effectively and efficiently satiated by the real-world objects that naturally satisfy them. The **ego**, which operates on the **reality principle**, identifies and pursues the real-world objects of instinctual drive and desire. However, the activity of the ego is constrained by the super-ego, which restricts the objects pursued to those that are socially approved (by the relevant parent).

These conflicts generate various forms of anxiety, notably neurotic and moral anxiety—feelings of being overwhelmed by instinctual drives and the shame and guilt associated with disapproved means of satisfying them. To explain how we avoid these forms of anxiety, Freud postulated a set of defense mechanisms employed by the ego, such as repression, identification, projection, and sublimation. Freud maintained that these defense mechanisms are unconscious, but play a major role in the explanation of normal as well as abnormal behavior (such as neurotic behavior). This was one of the distinctive features of his theory. For Freud, all aspects of human life, from the most bizarre dreams to the mundane choices of everyday life, are invested with symbolic meanings that express forms of wish fulfillment relating to early childhood sexual experiences and memories.

The basic elements of Freud's account of unconscious psychological states and processes had been anticipated by a number of earlier theorists. Leibniz and Fechner had acknowledged unconscious states; Mill and Helmholtz had recognized unconscious processes; and Herbart and Drobisch had maintained that some ideas are repressed. Nineteenth-century interest in the unconscious was evidenced by the popularity of Karl von Hartmann's (1842–1906) *Philosophy of the Unconscious* (1869), which ran into many editions. Schopenhauer and Nietzsche had claimed that much of human behavior is governed by irrational instinctual drives and desires, including sexual ones, and Janet had identified the role of pathogenic ideas in hysteria. Freud's genius lay in his integration of these various elements into a comprehensive theory of psychosexual development, personality, and motivation.

The Reception of Freud's Theory

Studies on Hysteria was well received, but the medical establishment dismissed Freud's theory of infant sexuality. However, this was not because it dealt with sexual matters (Sulloway, 1979), which were a lively topic of interest in the late 19th century, as evidenced by the popularity of works such as von Krafft-Ebing's *Psychopathia Sexualis* (1886), a compendium of case histories of sexual perversions, Albert Moll's *Perversions of the Sex Instinct* (1891), and Havelock Ellis's seven-volume *Studies in the Psychology of Sex* (1897–1928). The skepticism with which the medical community received Freud's theories was based partly upon his early advocacy of cocaine as a therapeutic agent, but also upon legitimate concerns

about the empirical adequacy of his theory—concerns reprised by many later critics of psychoanalysis.

In the early decades of the 20th century, a select group of physicians in Europe and America adopted Freud's theory. In 1902 Freud established a Wednesday Evening Group devoted to the discussion of psychoanalytic theory and case histories, which was reconstituted as the Vienna Psychoanalytic Society in 1908: Members included Alfred Adler (1870–1937), Abraham A. Brill (1874–1948), Sándor Ferenczi (1873–1933), and Carl Jung (1875–1961). There was sufficient interest in the psychoanalytic movement to support the First International Psychoanalytical Congress in 1908. Brill set up a psychoanalytic practice in New York in 1908 and produced the first English translations of Freud's work in 1909. Loyal disciples such as Ernest Jones (1879–1958) and Freud's daughter Anna Freud (1895–1982) helped create centers of classical psychoanalysis in Toronto, New York, and London.

However, the emerging psychoanalytic movement began to fragment as a result of the defection of Freud's former disciples, who promoted their own psychodynamic theories and therapeutic methods. Adler split in 1911 and developed a form of individual psychology based upon feelings of inferiority. Jung split in 1912 and developed a theory of the collective unconscious as the repository of symbolic meaning, which included but was not restricted to sexual meaning. Ferenczi rejected Freud's account of sexual wish fulfillment and championed the original seduction theory. Karen Horney (1885–1952), Melanie Klein (1882–1960), Erich Fromm (1900–1980), and Henry Stack Sullivan (1892–1949) developed theories that looked to the broader social and cultural contexts of human psychology and behavior as the source of psychodynamic conflict. Erik Erikson (1902–1994) developed a theory of psychosocial development that postulated crises of adjustment from infancy to old age. This fragmentation accelerated from the 1920s onward, when Freud began to drastically revise his own theories, to the point that one commentator could reasonably identify 36 distinct schools of psychoanalysis derived from the original Freudian orthodoxy (Harper, 1959).

In 1924 William Alanson White (1870–1937), then president of the American Psychiatric Association, urged psychiatrists to embrace psychoanalytic theory and practice, and psychoanalysis became the dominant theoretical orientation of most medically trained American psychiatrists in the first half of the 20th century (Routh & Reisman, 2003). However, American psychologists did not so readily embrace psychoanalysis, and their general response to Freud was respectful but critical (Green & Rieber, 1980; Hornstein, 1992). Hall, Münsterberg, and Witmer rejected psychoanalytic theory, although Hall admired Freud enough to invite him to speak at the 20th anniversary celebrations at Clark University in 1909.

Freud's American visit was a significant intellectual event. Accompanied by Brill, Jung, Ferenczi, and Jones, he delivered a series of lectures on psychoanalysis at Clark. These were well received and attended by American luminaries such

as Cattell, Goddard, James, Jastrow, Meyer, and Titchener. Freud's theory quickly became the focus of professional and public attention, displacing mesmerism, hypnotism, and other popular fads of the day (Green & Rieber, 1980). Dozens of articles were published in popular magazines and professional journals, leading the Harvard psychiatrist Morton Prince to complain that "Freudian psychology had flooded the field like a full rising tide and the rest of us were left submerged like clams in the sands at low water" (Hale, 1971, p. 434). However, most of the professional psychological response, such as the reviews by Woodworth (1917) and Jastrow (1932), remained critical. McDougall (1926) claimed that Freud's theory was brilliant but wrong, a common judgment among American psychologists. Meyer dismissed Freud's theory as dogmatic, and Prince developed Harvard psychiatry in effective opposition to psychoanalysis.

Freud's lectures at Clark won psychoanalysis a temporary place as a school of American psychology (Murchison, 1930). However, it was almost completely displaced within academic psychology during the behaviorist and neobehaviorist periods. Even the apparent exception, the attempt by neobehaviorists such as John Dollard (1900–1980) and Neal Miller (1909–2002) to integrate psychoanalysis and behaviorist learning theory (Dollard & Miller, 1950; Miller & Dollard, 1941), was little more than a reduction of Freudian concepts to those of conditioning theory (for example, transference was reduced to stimulus generalization). The discipline of clinical psychology that Lightner Witmer founded in 1907 developed independently of and often in opposition to the psychoanalytic movement, as clinical psychologists came to embrace theories and therapies grounded in behaviorist learning theory and cognitive psychology.

At the same time, Freud's theories came to have a significant impact beyond clinical medicine and psychology, when he extended them to society and religion in *Totem and Taboo* (1912–1913) and *The Future of an Illusion* (1927). They also came to have a significant impact on social sciences such as anthropology and sociology, postmodern and feminist critiques, and popular culture at large. This was especially true of American culture, where Freudian terminology quickly entered the vernacular. This latter development was not something that Freud would have welcomed. He was always ambivalent about America and feared that the commercially exploitive aspects of its culture would contaminate his theories. In 1935 he rejected a lucrative offer of \$100,000 from MGM to make a movie featuring psychoanalysis (Green & Rieber, 1980).

The Scientific Status of Freud's Theory

Freud always represented psychoanalysis as a scientific theory. However, the scientific credentials of his theory have been frequently questioned, as has Freud's integrity as a scientist. Doubts about his theory were raised almost from the moment

of inception—his first paper on hysteria received a frosty reception from fellow scientists, including sexual researchers such as von Krafft-Ebing—but more forcefully as the 20th century progressed. Scientific methodologists criticized Freud for interpreting data to suit his theory, for his tendency to dismiss or ignore alternative theories, and for his general reluctance to submit psychoanalytic theory to critical experimental tests (Cioffi, 1970; Popper, 1963).

There are certainly grounds for such complaints. Freud made the rather ridiculous claim that only psychoanalytically trained physicians could evaluate psychoanalytic theory, given the inevitable bias of untrained critics:

None but physicians who practice psychoanalysis can have any access whatsoever to this sphere of knowledge or any possibility of forming a judgment that is uninfluenced by their own dislikes and prejudices.

—(Freud, 1905/1953, Preface, p. 43)

He added insult to injury by suggesting that failure to accept psychoanalytic theory is a symptom of repression!

Freud did not appear to care much about the empirical evaluation of his theories. In 1934 Saul Rosenzweig (1907–2004) sent him reprints of his experimental studies of repression (Rosenzweig, 1933; Rosenzweig & Mason, 1934), but Freud's reply was dismissive and condescending:

I have examined your experimental studies for the verification of psychoanalytic propositions with interest. I cannot put much value on such confirmation because the abundance of reliable observations on which these propositions rest makes them independent of experimental verification. Still, it can do no harm.

—(Letter from Freud to Rosenzweig, cited in Rosenzweig, 1986, pp. 37–38)

Freud rarely took seriously alternative explanations of the clinical cases that provided the “reliable observations” on which psychoanalytic theory was supposed to be based. He explained Little Hans's fear of horses in terms of the projection of his fear of castration by his father (1909/1966, 10, 5–147), but dismissed the child's own account in terms of the fright he got the first time he saw a horse, when it fell down and caused a great commotion—a standard behaviorist explanation in terms of a classically conditioned fear (Wolpe & Rachman, 1960). He dismissed evidence of child abuse after he abandoned the seduction theory and developed the theory of the Oedipal and Electra conflicts. When Jeffrey Masson made this charge in the 1980s, he was promptly fired from his position at the Freud Archives (Masson, 1984).

Some scholars have tried to defend Freud against such charges (Glymour, 1980), and others have claimed that Freudian theory does license testable empirical predictions, some of which have been confirmed (Fisher & Greenberg, 1977;

Kline, 1972). Adolf Grünbaum (1983, 1984) has argued that Freud behaved like a “sophisticated scientific methodologist” in advancing arguments for psychoanalytic theory. Freud maintained that only psychoanalysis could attain a lasting and durable cure for neuroses, since only psychoanalysis could achieve the correct insight necessary for it:

After all, (a patient's) conflicts will only be successfully solved and his resistances overcome if the anticipatory ideas given (by his analyst) tally with what is real in him.

—(Freud, 1917/1963, 16, p. 452)

This claim, in conjunction with Freud's claim that psychoanalysis is an effective treatment of neuroses, constitutes what Grünbaum calls the **tally argument** for psychoanalytic theory.

Unfortunately for Freud's reputation and legacy, later research did not support these claims. Anna O's treatment is usually represented as an unqualified success, with all her symptoms eliminated. Yet in later years Anna, whose real name was Bertha Pappenheim (1859–1936), disputed this representation, claiming that she had to be admitted to a sanitarium for treatment in 1882, shortly after she completed her “talking cure” with Freud and Breuer. Pappenheim was a social worker and pioneer of women's rights, who founded schools and homes for disadvantaged young girls. She was critically dismissive of psychoanalysis as a theory and therapy and refused to allow any of those in her care to be exposed to it (Ellenberger, 1972).

In a famous study, Hans Eysenck (1916–1997) claimed that the recovery rate for neurotics who receive psychoanalytic therapy is not significantly superior (statistically) to the “spontaneous remission” rates of neurotics who do not (Eysenck, 1952). In response, the American Psychoanalytic Association commissioned its own study of the efficacy of psychoanalysis, but the results were so bad that the association suppressed its publication (Storr, 1966). Later studies indicated that other forms of psychological therapy, such as behavior therapy, achieve recovery rates comparable if not superior to those of psychoanalysis (Eysenck, 1965; Rachman & Wilson, 1971) and that correct insight is not necessary for effective psychotherapy (Wallerstein, 1995). Such findings did considerable damage to the reputation of psychoanalysis among psychologists. The tally argument was Freud's response to the charge that the efficacy of psychoanalysis is best explained in terms of suggestion, commonly held to be the basis of earlier mesmeric and hypnotic treatments. Unfortunately, the charge remains unanswered to the present day (Greenwood, 1996, 1997).

Freud's theory of unconscious psychological states and processes was a critical target for behaviorists from Watson to Skinner, although there was nothing inherently unscientific about it. Karl Pribram and Merton Gill have argued that the basic theoretical systems postulated by Freud in his *Project for a Scientific Psychology*

have been confirmed by contemporary cognitive psychology and neurophysiology (Primbram & Gill, 1976).

Freud died in London in 1939, after escaping from the Nazis in 1938 with his daughter Anna; his four sisters perished in the concentration camps. He left a major intellectual and cultural legacy and a legion of psychodynamic schools within medical psychiatry. However, by this time mainstream American psychologists, including clinical psychologists, had largely abandoned his theories.

SCIENTIFIC PSYCHOLOGY AND ABNORMAL PSYCHOLOGY

The development of psychology in Germany in the late 19th century promoted a scientific approach to the assessment and treatment of psychological disorders, despite the fact that Wundt expressed little interest in this area (although he published a paper on hypnosis and suggestion in 1888). Early German abnormal psychology was dominated by Wilhelm Griesinger (1817–1868), the founder of academic psychiatry in Germany, and author of *Mental Pathology and Therapeutics* (1845). Griesinger claimed that since mental disorders are brain diseases, they are the proper subject matter of general medicine. He championed this position in the first issue of *The Archives for Psychiatry and Nervous Diseases*, which he founded in 1868:

Patients with so-called mental diseases are really individuals with diseases of the nerves and the brain. . . . Psychiatry . . . must become an integral part of general medicine and accessible to all medical circles.

—(1868, p. 12)

Griesinger's reductive neurophysiological conception of psychopathology matched the reductive physicalism of the Berlin Physical Society and was championed by Henry Maudsley (1835–1918) in Britain and by Valentine Magnan (1835–1916) in France.

The German psychiatrist Karl Ludwig Kahlbaum (1828–1899) developed a classificatory system based upon the course and outcome of psychological disorders, rather than their neuropathology. Emil Kraepelin (1856–1926), a student and life-long friend of Wundt's, who founded laboratories of experimental psychopathology at Heidelberg and Munich, developed a classificatory system that combined the developmental and neurological approaches of Kahlbaum and Griesinger and was avowedly based upon the principles of the new physiological psychology (Kraepelin, 1883). He stressed the importance of identifying the core symptoms of psychological disorders, but held that the only way to discriminate between disorders with similar symptoms was through the developmental analysis of their course and outcome.

Kraepelin's classificatory system, originally published in *Compendium of Psychiatry* (1883) and revised in multivolume editions of his *Textbook of Psychiatry* (1915), included familiar categories such as manic-depressive psychosis and paranoia. It also included novel categories such as dementia praecox, later characterized as **schizophrenia** by the Swiss psychiatrist Eugene Bleuler (1857–1939). Kraepelin (following Wundt) treated dementia praecox as a disorder of attention and maintained that it was a constitutional disease incapable of treatment, although he later admitted that around 10 percent of diagnosed cases make an almost full recovery (Reisman, 1991). In contrast, Bleuler conceived of schizophrenia as a disorder based upon the disassociation of thought and emotion and maintained that it could be treated (Bleuler, 1911/1950).

Kraepelin revised his classification system up until his death in 1926, and aspects of it were later incorporated in the *American Standard Nomenclature of Disease* and the *International Classification of Disease*. In 1952 American psychiatrists established their own diagnostic criteria in the *Diagnostic and Statistical Manual of Mental Disorders* (DSM), published by the American Psychiatric Association. The original manual defined psychological disorders in terms of their theoretical causal etiology (DSM, 1952), but this was abandoned in later editions such as DSM II (1968), DSM III (1980), DSM III-R (revised, 1987), DSM IV (1994), and DSM IV-TR (2000), in which classifications were based upon essential and common symptoms (Mayes & Horwitz, 2005; Wilson, 1993).

Other psychiatrists and psychologists adopted Kraepelin's scientific approach to abnormal psychology. In America, psychological laboratories were set up at medical facilities such as the McLean Hospital in Massachusetts, the Worcester State Hospital, and the New York State Psychiatric Institute (Maher & Maher, 2003). However, while the number of hospitals dedicated to the care of the psychologically abnormal expanded dramatically in the early decades of the 20th century—from around 100 at the turn of the century to between 200 and 300 by the 1930s, with a resident population of over half a million—the control and treatment of patients suffering from psychological disorders remained firmly in the hands of medically trained professionals.

In the 19th century, Americans shared the European enthusiasm for electrical and magnetic treatments, as evidenced by the popularity of the Reverend John Dods's *Electrical Psychology* of 1850. Mesmerism was originally promoted in America by traveling French demonstrators such as Dr. Joseph du Commun, who arrived in 1815 and started a society of magnetizers in New York (Beloff, 1975). For many Americans, mesmerism was associated with spiritualism, because they believed that the somnambulant had clairvoyant powers. Phineas Quimby (1802–1866), a former clockmaker, was one of the first Americans to employ mesmeric techniques for therapeutic purposes: His most famous patient was Mary Patterson (later Mary Baker Eddy), the founder of Christian Science (Beloff, 1975). Mesmerism was also closely associated with religion and formed the basis of the

Emmanuel Movement, a Christian ministry devoted to the treatment of nervous disorders that James christened the “mind-cure” movement. Elwood Worcester (1864–1940), a student of Wundt’s, and the physician Isador Coriat (1875–1943), an interpreter of Freud, founded the movement, which was named after Emmanuel Church in Boston, where the original meetings were held.

As academic and applied psychology developed in America, it was initially only medically trained psychologists who concerned themselves with abnormal psychology and psychotherapy. William James, who had originally trained as a physician, took an active interest in the work of Charcot and Janet (both of whom he visited) and was greatly impressed by Breuer and Freud’s *Studies on Hysteria* when it appeared in 1895. He began to employ hypnosis in his own therapeutic practice and maintained that hypnosis was the best treatment for nervous diseases such as hysteria. He also employed hypnosis in his analysis of mediums, cementing the early association of mesmeric techniques with spiritualism and religion, which led Joseph Jastrow to lament in his 1901 APA presidential address that psychologists were too often identified with “spook chasers” (Reisman, 1991, p. 47).

Hugo Münsterberg, the medically trained student of Wundt’s who took over the psychology laboratory at Harvard from James, employed a variety of directive and suggestive therapeutic techniques, including hypnosis, in his early attempts at psychotherapy. Boris Sidis (1867–1923), a medically trained student of James’s, became director of the New York State Psychiatric Institute. In *The Psychology of Suggestion* (1898), he characterized the unconscious mind as uncritical, irrational, amoral, involuntary, animalistic, and suggestible. He employed hypnosis in his treatment of hysterical patients for a number of years, but later abandoned it in favor of a form of therapy based upon sympathetic rapport (Sidis, 1902).

Morton Prince (1854–1929), another medically trained student of James’s who studied with Charcot in Paris and Bernheim in Nancy, played a significant role in the early institutional development of abnormal psychology in America. Prince founded the *Journal of Abnormal Psychology* in 1906, the Psychopathological Association (with Sidis) in 1910, and the Harvard Psychological Clinic in 1927. He is perhaps best remembered for his study of dissociation, which he followed Janet in treating as a form of hysteria. In 1906 he published *The Dissociation of Personality*, an analysis of his treatment of Christine Beauchamp, a classic case of “multiple personality.” Prince was impressed by Freud’s work and helped to introduce psychoanalysis to America, although he developed Harvard psychiatry in opposition to psychoanalytic theory and practice. He was an early proponent of a view that proved enormously popular with later generations of clinical psychologists. Prince claimed that psychological disorders are the product of maladaptive learning, which could be relieved through relearning or “education” (Prince, 1909–1910).

Adolf Meyer (1866–1950), who completed his medical training in Zurich and took up a position at the Illinois Eastern Hospital for the Insane from 1893 to 1895, shared this view. He was later appointed director of clinical research at

Worcester Hospital for the Insane, with an academic appointment at Clark University (he took over from Hall as instructor in psychology at the hospital). Meyer later became professor of psychiatry at Johns Hopkins and director of the Phipps Psychiatric Clinic in Baltimore, where he integrated psychological and biological approaches in the medical training of psychiatrists. He encouraged Watson to work with infants and pioneered the use of developmental case histories. Meyer dismissed Freud's theories, but also rejected Griesinger and Kraepelin's claim that most psychological disorders are products of neural pathology. He treated them as functional disorders: the product of maladaptive responses rather than constitutional degeneracy. Meyer maintained that many psychological disorders, including dementia praecox or schizophrenia, are ineffective "psychobiological reactions" to the difficulties and stresses of everyday life, which lead to the formation of "progressive habit deteriorations" (Meyer, 1912). He claimed that they could be treated through training programs directed toward more adaptive and effective responses.

Meyer was a supporter of the **mental hygiene movement**, originally inspired by the work of Clifford Beers (1876–1943), who founded the first mental hygiene society in Connecticut in 1908. Beers was a former suicidal depressive who described his experience in mental institutions in *The Mind That Found Itself* (1908), with an introduction by William James. The aim of the movement was to increase public awareness of mental illness and promote its effective treatment. Henry Phipps, the Baltimore industrialist, was inspired by Beer's work to endow the psychiatric clinic at Johns Hopkins, later known as the Phipps Clinic. Meyer served as its first director and helped Beers set up the National Committee on Mental Hygiene in 1909, dedicated to the public dissemination of information about the prevention and treatment of mental disorder.

Lightner Witmer, generally recognized as the founder of clinical psychology in America, was highly critical of the therapeutic practices of some of his medically trained psychological colleagues. Conversely, a good many medically trained psychiatrists were skeptical about the therapeutic pretensions of their psychological colleagues. When the psychiatrist Karl Menninger (1893–1990) founded the American Orthopsychiatric Association in 1924, he originally restricted the membership to psychiatrists, although it was later extended to clinical psychologists and other mental health professionals such as Witmer and Goddard.

The work of clinical psychologists in the first four decades of the 20th century was largely devoted to mental testing and diagnosis, remedial education, and forms of training designed to redirect maladaptive behavior. They developed psychometric measures of personality and psychological functioning, such as the Downey-Will Temperament Test, Woodworth's Psychoneurotic Inventory, Symonds Adjustment Questionnaire, and the Woodworth-House Mental Hygiene Inventory. David Levy promoted the use of the Rorschach personality test in America, based upon the inkblot pictures first described by Hermann Rorschach

(1884–1922) in *Psychodiagnostik* (1921). Christiana D. Morgan and Henry Murray introduced the Thematic Apperception Test (TAT), another measure of personality, in 1935. Clinical psychologists developed diagnostic tests such as the Babcock Deterioration Test and eventually won the right to make committals to state institutions and hospitals on the basis of such tests. However, until the Second World War the treatment of psychological disorder remained the exclusive domain of medically trained psychiatrists, who supervised clinical psychologists in hospitals and clinics.

ECT, LOBOTOMY, AND PSYCHOPHARMACOLOGY

As the 20th century advanced, new forms of therapy displaced the eclectic mix of treatments popular in the 19th century. **Electroconvulsive treatment (ECT)**, in which convulsions are induced by passing an electrical current through the brain, was introduced in the late 1930s, although the use of electrical stimulation as a treatment goes back to antiquity, and camphor-induced seizures had been used as a treatment for centuries. The modern treatment regime was developed by Joseph Ladislau von Meduna (1896–1964), a Hungarian physician who used pentylene-tetrazol (metrazol), a synthetic extract of camphor, to artificially induce convulsions in patients suffering from schizophrenia. He reported qualified success and later extended this form of treatment to melancholics and manic-depressives (Meduna, 1935). Ugo Cerletti and Lucio Bini, two Italian psychiatrists who had read reports of Meduna's work, created convulsions in patients by passing an electrical current through their temporal lobes and also reported qualified success (Cerletti & Bini, 1938). Cerletti and Bini first used this treatment on a vagrant who had been brought to them by the police, after he had been found wandering the streets of Rome speaking "incomprehensible gibberish" (Impasato, 1960).

Metrazol shock treatment was widely used in psychiatric hospitals in Europe and America until the 1940s, but was eventually displaced by ECT, which Lothar Kalinowski and Renato Almansi introduced to America in 1939. Originally employed in the treatment of schizophrenia, ECT was later extended to the treatment of forms of depression, particularly those that did not respond well to drug treatments. The use of ECT has been controversial since its inception. Despite the multitude of theories, there is no generally accepted account of its efficacy (Sackheim, 1988), and the often debilitating side effects, from memory loss to brain damage, have led many to question its therapeutic utility. The use of ECT declined during the 1970s, but appears to be on the increase again, although not because of any major developments in theoretical validation or improvement in treatment outcomes (Giles, 2002).

Another controversial treatment that was introduced in the 1930s was **prefrontal lobotomy**, a surgical operation producing lesion of the nerve fibers in

the frontal lobes (severing the fiber tracts connecting the prefrontal lobes with the thalamus). The Portuguese physician Egas Moniz (1874–1955), whose original idea had been to inject the frontal lobes with alcohol, pioneered the use of the technique. Although he was disappointed to discover that the surgery did not eliminate chronic delusions in schizophrenics, as he had originally hoped, Moniz claimed that it changed their emotional response from distress to apathy (Moniz, 1937). Reports of the success of these operations were greatly exaggerated and neglected their side effects. Nevertheless, the technique was widely adopted in Europe and America in the late 1940s and early 1950s, when it was extended to the treatment of other psychological disorders (Valenstein, 1986). Moniz won the Nobel Prize for his work in 1949, but it brought him little satisfaction. He was shot by one of his lobotomized patients and remained a paraplegic for the rest of his life.

Walter Freeman and J. W. Watts promoted the operation in the United States (Freeman & Watts, 1942). Freeman reputedly performed over 3,000 lobotomies in his lifetime and championed the surgery as the solution to America's social ills. However, the use of the procedure declined in the late 1950s amid concerns about its serious side effects and ethical questions about the conditions under which lobotomies were performed—sometimes physicians unqualified in neurosurgery conducted them in their offices. Freeman, who once performed the procedure in a motel room, lost his surgical privileges following the death of a lobotomy patient during an operation (Maher & Maher, 2003). As in the case of ECT, lobotomy was employed for many years in the absence of any generally accepted theoretical justification and amid serious doubts about its efficacy. The underlying motivation for continuing both treatments seems to have been to maintain the morale of professional mental health practitioners by doing *something* for their patients (Reisman, 1991).

Psychoactive Drugs and Institutional Care

More effective and enduring was the employment of psychoactive drugs. The tranquilizers chlorpromazine and reserpine (which Avicenna had used in its herbal form *rauwolfia*) were employed in the treatment of schizophrenia, and lithium and other MAO (monoamine oxidase) inhibitors in the treatment of mania and depression. The theoretical rationale for these and later drug treatments was that psychological disorders are related to the disruption of the balance of chemical neurotransmitters in the brain, such as norepinephrine, dopamine, and serotonin, which the various drug treatments aimed to restore—an old idea in modern neurophysiological guise. These treatments did reduce the severity and frequency of symptoms, but they also created dependencies, and critics complained that they simply managed rather than relieved psychological disorders. Although they

transformed the lives of many schizophrenics, schizophrenia remained a debilitating condition for others.

The success of psychoactive drug treatments had a significant impact on institutional care. Many schizophrenics found release from their catatonic states, and restraints were removed from formerly violent patients. In many cases they obviated the need for institutional care; patients who had formerly required hospitalization could now be treated on an outpatient basis. This led to a dramatic drop in the population receiving institutional care, which fell from over 600,000 in the 1940s to less than 150,000 by the 1970s (Reisman, 1991). The process of deinstitutionalization was accelerated by the social policies of the 1960s. The Community Mental Health Centers Act of 1963 provided federal support for community centers offering outpatient mental health services, along with outreach and drug and alcohol treatment programs.

Unfortunately, the community mental health center program was seriously underfunded, and former patients were not always able to manage their drug regimes. The process of deinstitutionalization came to be driven as much by economic as by professional psychological goals, and many of those suffering from psychological disorders joined the populations of homeless persons in major metropolitan centers. The problem was compounded by legal challenges to the committal authority of psychologists and psychiatrists, enabling potentially violent and self-destructive individuals to refuse institutional treatment, including drug and ECT treatment. One of the saddest outcomes of this well-intentioned social experiment was a new generation of Tom o' Bedlams on the streets of New York, London, and Rome, begging on subways and muttering to themselves on city sidewalks.

The Myth of Mental Illness

In 1960 the psychiatrist Thomas Szasz published an article and book with the title "The Myth of Mental Illness." He claimed that most neuroses and psychoses are "problems of living" rather than pathological psychological disorders, although he did recognize that some have their origin in neurological damage and dysfunction. He argued that the treatment of patients as "mentally ill" sanctioned illegitimate exercises of social control: Patients were imprisoned, institutionalized, and subjected to drug, surgical, and behavioral treatments against their will. Szasz claimed that such involuntary treatments were never justified and constituted "crimes against humanity." His views promoted legal challenges that resulted in more stringent conditions and limits on the involuntary incarceration and treatment of psychiatric patients.

Szasz's critique of the moral presumptions of psychiatry and his claim that many treatment practices undermine fundamental principles of individuality and

freedom found a responsive audience in the 1960s, as evidenced by the popularity of novels and films such as Ken Kesey's *One Flew Over the Cuckoo's Nest*. However, most of the psychiatric establishment dismissed Szasz as a dangerous radical, in the same league as Timothy Leary, the former Harvard physician turned LSD guru. While damaging to psychiatry, Szasz's critique provided indirect legitimization for clinical psychological treatments, particularly those based upon behaviorism and cognitive psychology, since it was fairly easy to represent "problems in living" as problems of learning and cognitive adjustment.

POSTWAR CLINICAL PSYCHOLOGY

In the First World War, psychologists played only a minor psychotherapeutic role. The applied psychologist Harry Hollingworth (1880–1956) of Barnard College examined soldiers suffering from "shell shock" while serving (with the rank of captain) in the army hospital at Plattsburgh, New York (Hollingworth, 1920), but most psychologists were assigned to personnel selection and intelligence testing. Between the wars clinical psychologists continued to focus on psychometrics and remedial education and generally deferred to psychiatrists in the treatment of psychological disorders. The training of clinical psychologists was haphazard and unregulated, despite the efforts of those clinical and counseling psychologists who petitioned the APA for the development of professional training standards. While some psychology internships were established as part of clinical training (the first at Vineland in 1908), students participating in them were generally restricted to research and psychometric examination (Routh, 2000).

After the Second World War, clinical psychologists continued to develop diagnostic and personality tests, such as the Minnesota Multiphasic Personality Inventory (MMPI), but became more directly engaged in the treatment of psychological disorders, including depression and schizophrenia. This was largely a function of their war experience and the huge demand for clinical services in the aftermath of the war. Although many psychologists were employed in personnel evaluation and assessment during the war, many were also employed in counseling and psychotherapy (Hunter, 1946), since the number of psychological casualties overwhelmed the resources of psychiatry. Consequently, many postwar clinical psychologists came to believe that the provision of psychotherapy was an integral part of their professional role (Krugman, 1945).

When the United States entered the Second World War in December 1941, the government recognized that there were too few psychiatrists to service the psychological needs of soldiers and directed the U.S. Public Health Service (USPHS) and the Veterans Administration (VA) to increase the availability of clinical psychologists. After the war, there was a huge demand for clinical psychologists to accommodate the psychological needs of the 16 million returning veterans (along with

the 4 million from previous wars). This led to the rapid development of university training programs, funded by the USPHS, the VA, and the National Institute of Mental Health (founded in 1948). The APA worked with these institutions to establish a set of standards for clinical training and a program of accreditation, first established for clinical psychologists in 1946 and for counseling psychologists in 1952. By 1949, there were 42 graduate schools offering a doctorate in clinical psychology, and about 149 offering some form of clinical training (Reisman, 1991). The numbers of clinical psychologists within the profession began to increase dramatically and was reflected in the divisional affiliation of the presidents of the APA. Carl Rogers was elected president in 1946, the first of many clinical psychologists to hold the office after the war. The Division of Clinical Psychology (Division 12) constituted the largest division of the reconstituted postwar APA, and by 1954 its 1,500 members outnumbered the combined membership of the Divisions of Experimental and General Psychology by two to one (Capshe, 1999).

Clinical Training

The 1947 APA Committee on Training in Clinical Psychology recommended that graduate training programs in clinical psychology should include a solid grounding in theoretical and experimental psychology, with internships and practical training in both diagnosis and therapy: “at least a four-year program which combines academic and clinical training throughout but which includes intensive clinical experience in the form of an internship” (1947, p. 544). This combination of academic research and practical training became known as the **scientist-practitioner** or “Boulder” model of clinical training, after it was officially endorsed at the APA Conference on Clinical Training in Psychology, funded by the Public Health Service and held in Boulder, Colorado, in 1949.

A later APA conference, held at Vail, Colorado, in 1973, reaffirmed the scientific-practitioner model, but also endorsed an alternative “practitioner” model, which stimulated the development of clinical programs offering a Doctor of Psychology degree (PsyD), specifically oriented to professional clinicians. Leta Stetter Hollingworth had suggested such a degree in 1918, but it was not instituted until 1968, when the University of Illinois allowed clinical graduate students to pursue either a PhD or PsyD, a practice followed by other graduate schools in the ensuing decades. The alternative practitioner model was also supported by the creation of independent professional schools of psychology, such as the California School of Professional Psychology, founded in 1969 (Routh & Reisman, 2003). By the 1980s there were about 20 professional schools, although some remained affiliated with universities, such as the Rutgers Graduate School of Applied and Professional Psychology.

In the postwar period, the theories and therapies that clinical psychologists developed reflected the influence of behaviorism and cognitivism within academic

psychology. Joseph Wolpe (1958) developed “systematic desensitization” as a means of relieving phobias, a form of treatment that had been pioneered by Mary Cover Jones in her extinction of Peter’s fear of rabbits (Jones, 1924) and documented by Locke in *Some Thoughts Concerning Education* (1693/1989). Skinnerian operant-conditioning techniques became almost as popular as drug treatments in many hospital wards, with therapists shaping the behavior of patients through token economy programs (Stahl & Leitenberg, 1976). Although legal challenges eventually restricted the use of such programs and aversion therapy, behavioral treatments remained popular among clinical psychologists. The advent of the cognitive revolution in psychology introduced a variety of cognitive and cognitive-behavioral theories and therapies (Bandura, 1977; Beck, 1972; Rotter, 1975; Seligman, 1975).

HUMANISTIC PSYCHOLOGY

Abraham Maslow (1908–1970) and Carl Rogers (1902–1987) raised doubts about scientific approaches to psychological disorders, largely in reaction to some of the more technologically oriented behaviorist treatments. They developed an alternative approach that came to be known as **humanistic psychology**. Maslow was a former behaviorist who came to believe that the behaviorist approach was too narrow, possibly through his contact with European refugees such as Adler, Lewin, and Wertheimer in New York City. He developed his theory of personality as a form of **self-actualization** through an intensive study of the lives of Wertheimer and Ruth Benedict, two especially creative individuals whom he greatly admired. Maslow later maintained that Wertheimer was the inspiration for his theory.

Maslow held that human motivation is grounded in the capacity of all individuals to actualize their potential, although in his own research he focused on individuals who seemed to live especially creative and rewarding lives. Maslow claimed that human motivation is arranged in a hierarchy and that basic human needs, such as the need for security and love, have to be satisfied before people can become true “self-actualizers” and attain their full potential. Few people attain full self-actualization in practice, because they are too engaged with the satisfaction of basic needs or lack the courage and energy to develop their true selves (Maslow, 1943). However, Maslow claimed that all humans have the capacity to enrich their lives by actualizing their higher potential, even if they cannot attain the creative heights of a Wertheimer or Benedict. He developed his brand of humanistic psychology as a “third force” in psychology, in opposition to the dehumanizing scientific objectivity of behaviorism and the “crippled psychology” of psychoanalysis (Maslow, 1954, p. 180). Maslow stressed the values of autonomy and choice in the development of a full human life, including the open expression of feeling. He was elected president of the APA in 1967, but left academia the following year for

a research fellowship at the Saga Corporation, where he remained until his death in 1970.

Carl Rogers developed what came to be known as “person-” or **client-centered therapy**, a form of psychotherapy that champions the self-knowledge of the patient and the empathy of the therapist over more detached scientific approaches (Rogers, 1947, 1951, 1966). Like Maslow, Rogers affirmed that all individuals are capable of actualizing their highest potential and that their attempts may be promoted or impeded through personal development and social interaction. He claimed that individual self-fulfillment requires acknowledgment of the unconditioned worth of the person by parents, friends, and therapists. Like Maslow, he lauded human autonomy and spontaneous expression over the constraints of scientific objectivity, although his own research was focused on emotionally disturbed individuals rather than creative self-actualizers. Rogers was elected president of the APA in 1946 and later received its Distinguished Scientific Contribution and Distinguished Professional Contribution Awards.

The roots of humanistic psychology lie in Brentano’s phenomenological psychology and philosophy, especially as developed by Edmund Husserl and Martin Heidegger (1889–1976), which focused upon the phenomenological exploration of consciousness and being as the goal of self-knowledge. One branch of phenomenology developed as existential philosophy and psychology, which emphasized the creative role of the individual in the determination of his or her essential being, through the work of Maurice Merleau-Ponty (1908–1961), Ludwig Binswanger (1881–1966), and Jean-Paul Sartre (1905–1980). Rollo May (1909–1994) promoted existential psychology in America and extended basic existentialist principles to psychotherapy and personality theory. Amedeo Giorgi (1931–) claimed that existential psychology is the foundation of a truly human science (Giorgi, 1970). Giorgi was a member of the psychology department at Duquesne University, which became the center of existential-phenomenological psychology in the United States and sponsor of the *Journal of Phenomenological Psychology*. The cause of humanistic psychology was also advanced by the development of personality psychology, which stressed the individuality and variability of personality and resisted the fashion for operationalized theory and rigorous experimentation, notably through the pioneering work of William Stern (1935, 1938) and Gordon Allport (1937, 1955).

Humanistic psychology was a popular movement in psychology throughout the 1960s and 1970s. *The Journal of Humanistic Psychology* was founded in 1961, the American Association for Humanistic Psychology in 1962, and the APA Division of Humanistic Psychology in 1971. Yet although it was respectfully acknowledged and institutionally recognized, few mainstream experimental or clinical psychologists embraced it. The empirical foundation for humanistic psychology was always doubtful, and humanist psychologists never developed a common theoretical or metatheoretical position, as they readily acknowledged. They often claimed that

they were not opposed to scientific approaches per se and suggested that these could be “incorporated” within humanistic psychology. However, they had great difficulty in squaring their avowed commitment to causal determination with their hymning of human freedom and autonomy, although Joseph Rychlak made a heroic effort to incorporate humanistic goals within a scientific perspective in works such as *The Psychology of Rigorous Humanism* (1988). Maslow and Rogers lauded the Nietzschean free expression of emotion, but other humanistic psychologists were inspired by the Platonic ideal of the rational control of emotion, as in the case of Albert Ellis’s promotion of **rational-emotive therapy** (Ellis, 1958).

INTO THE 21st CENTURY

The introduction of certification and licensing laws extended the professional authority of clinical psychologists. The first certification laws were introduced in Connecticut in 1945, requiring a doctoral degree and a year or more of professional experience. The next year the APA created the American Board of Examiners in Professional Psychology (ABEPP) to ensure the maintenance of national standards in clinical psychology and counseling. Clinical psychologists had earlier won the right to certify patients for institutional and hospital care and to serve as medical staff in hospitals. They later won the right to treat patients and receive third-party insurance payments, as a result of lawsuits such as *Blue Shield of Virginia vs. McCready* (1982).

However, the postwar advancement of the profession was not matched by obvious advances in fundamental theory and therapy. Alternative cognitive, behavioral, neurophysiological, developmental, and constitutional theories of psychological disorder abounded, with little prospect of integration or accommodation. The situation was compounded by disagreements about the scientific classification of psychological disorders and the historical expansion of diagnostic categories. *The Diagnostic and Statistical Manual of Mental Disorders* (DSM) recognized 106 categories when it was first published in 1952. The 1968 edition (DSM-II) recognized 182 categories; by the 1980 edition (DSM III) the number had expanded to 265. Such category expansion did not always appear to be a natural consequence of theoretical refinement. The revised third edition, which appeared in 1987 (DSM-III-R), was judged by many to be inferior to the original third edition.

Forms of psychological therapy continued to proliferate in the postwar period. By the end of the 1970s, Beutler (1979) estimated that there were around 130 different schools of psychological treatment. Whatever their theoretical rationale, they all came to be haunted by the type of critique originally advanced against psychoanalysis by Hans Eysenck (1952), who maintained that the recovery rates for neurotics receiving psychoanalytic treatment were no better than the “spontaneous

remission" rates of those left untreated. Around the same time Meehl (1954) complained that clinical psychologists were poor diagnosticians, whose rates of diagnostic success were no better than those based upon statistical actuarial tables relating symptoms and disorders, a charge repeated by Kleinmuntz (1967) with respect to clinicians' interpretations of MMPI, Rorschach, and TAT scores.

Later comparative studies of professional psychological treatments with "no-treatment" groups did suggest that they were effective (Sloane et al., 1975), and later meta-analyses (Luborsky, Singer, & Luborsky, 1975; Smith & Glass, 1977; Smith, Glass, & Miller, 1980) suggested that virtually all forms of psychological treatment are better than no treatment. However, such studies also indicated that most forms of psychological treatment are about equal in efficacy and that none are demonstrably more effective than placebo control treatments. This was a troubling outcome, given the impoverished nature of most placebo control treatments, especially with respect to "nonspecific" factors such as client and therapist expectancy and credibility of treatment (Borcovec & Nau, 1977; Kazdin & Wilcoxon, 1978), which some suggested might account for the efficacy of all forms of psychological treatment (Bergin & Lambert, 1984; Frank, 1974, 1983).

Concerns about the theoretical justification of professional psychological treatments were exacerbated when Congress demanded scientific evidence for them as a condition for endorsing insurance payments to clinical psychologists. Yet despite the plethora of later studies, including the massive National Institute of Mental Health study of depression in the 1990s (Elkin, 1994; Elkin et al., 1989), the suggestibility of clients (and therapists) remains a viable explanation of the efficacy of many forms of psychological treatment (Greenwood, 1997), the type of explanation that Freud's tally argument was designed but failed to refute.

Despite the initial rapport between clinical psychologists and medical psychiatrists, tensions developed as the century progressed, especially with the postwar intrusion of clinical psychologists into the realm of psychotherapy. The profession of medical psychiatry resisted the certification of clinical psychologists and challenged their right to provide psychotherapy and receive insurance payments for their services. They continue to resist the extension of prescription privileges to clinical psychologists (DeLeon & Wiggins, 1996).

Tensions also increased between clinical practitioners and academic psychologists when clinical, consulting, and other professional psychologists came to outnumber the mainstream academic members of the APA in the postwar period. Having for many years been treated as second-class citizens by the academicians, clinical psychologists came to dominate and control the institutions of the APA, including the presidency and the governing committees. They refused to surrender their new powers to the academics, who broke away to form the American Psychological Society (APS) in 1988. Thus at the beginning of the 21st century, the APA is primarily a professional association, dominated by clinical and other applied psychologists, and will likely remain that way for the foreseeable future (Pickren & Fowler, 2003).

DISCUSSION QUESTIONS

1. Why do you think the history of clinical psychology is so presentist? Does the progress of 20th-century clinical psychology justify this stance?
2. To what degree did Janet anticipate Freud? Do you think Janet was justified in accusing Freud of plagiarism?
3. Do you think psychoanalysis and behaviorism can be reconciled, as Dollard and Miller (1950) claimed?
4. Freud maintained that those who reject psychoanalytic theory are repressed. Would it be fair to respond that those who accept it are engaged in wish fulfillment?
5. Can “mental illness” really be characterized as a “myth”?
6. Should clinical psychologists be both scientists and practitioners?

GLOSSARY

abreaction In psychoanalysis, the cathartic expression of emotionally charged memories.

alienist Early name for a physician engaged in the diagnosis and treatment of the mentally disturbed.

dementia praecox Early name for disorder later characterized as schizophrenia, so-called because of its early onset and rapid progression.

client-centered therapy Form of psychotherapy based upon acknowledgment of the intrinsic worth of people, in contrast to more detached scientific approaches.

countertransference Emotional attachment that a therapist develops for a patient in the course of psychoanalysis.

ego In psychoanalytic theory, aspect of personality that identifies and pursues the real-world objects of instinctual drive and desire.

electroconvulsive treatment (ECT) Controversial treatment of schizophrenia and depression in which convulsions are induced by passing an electrical current through the brain.

Emmanuel Movement Christian ministry devoted to the treatment of nervous disorders.

free association In psychoanalysis, method of identifying pathogenic ideas by having the patient relax and describe whatever comes into his or her mind, no matter how apparently trivial or potentially embarrassing it might be.

- Freudian slip** Symbolic wish fulfillment expressed in everyday accidents and errors.
- humanistic psychology** An approach to psychological disorders and their treatment that emphasizes the autonomy, potential, and personal feelings of individuals, in contrast to objective scientific approaches.
- hypnosis** Term coined by the Scottish surgeon James Braid to describe the state of “nervous sleep” induced by mesmeric treatments.
- id** Instinctual core of personality, comprising basic drives and desires (such as hunger, thirst, and sexual desire).
- latent content** Repressed content (of dreams).
- manifest content** Apparent content (of dreams).
- mental alienation** Early term used to describe the troubled condition of those suffering from psychological disorders.
- mental hygiene movement** Early-20th-century movement that aimed to increase public awareness of mental illness and promote its effective treatment.
- mesmerism** Form of treatment developed by Franz Mesmer, supposedly based upon the manipulation of magnetic forces in the body.
- moral management** Form of 19th-century psychological treatment directed to the transformation of character through personal counseling and religious training.
- Nancy school** The theoretical position developed by Nancy physicians Ambroise-Auguste Liébault and Hippolyte Bernheim, who claimed that suggestion is the primary vehicle of hypnotism and that suggestibility is a normal and universal psychological trait.
- neuroses** Term first used by the Edinburgh physician William Cullen to characterize physical or psychological disorders caused by damage to or disease of the nervous system.
- neurasthenia** Nervous exhaustion.
- Oedipus complex** In psychoanalytic theory, emotional conflict generated by the male child’s sexual desire for his mother and fear of his father, resolved through identification with his father.
- paranoia** Term employed by Johann Christian Heinroth to describe a variety of disorders based upon intellectual and emotional disturbance.
- pleasure principle** In psychoanalytic theory, principle directed to the gratification of basic drives and desires.
- prefrontal lobotomy** Surgical operation producing lesion of the nerve fibers in the frontal lobes, employed as a treatment of schizophrenia and other disorders.
- psychiatry** Term introduced by Johann Christian Reil to describe the study and treatment of mental disorder.

rational-emotive therapy Form of therapy based upon the rational control of emotion.

reality principle In psychoanalytic theory, principle directed to the identification and pursuit of real-world objects of instinctual drive and desire.

schizophrenia Term introduced by Eugene Bleuler to describe the disorder formerly classified as dementia praecox, which he believed was based upon the dissociation of thought and emotion.

scientist-practitioner model The model of clinical training combining academic research and practical training that was endorsed at the APA conference on clinical training in psychology in Boulder, Colorado, in 1949.

seduction theory Freud's early theory that neurotic symptoms are the product of repressed memories of childhood sexual abuse.

self-actualization In Maslow's personality theory, the capacity every individual has to reach his or her highest potential.

super-ego In psychoanalytic theory, the aspect of personality based upon the internalization of the moral principles of the parent of the opposite sex, which restricts the objects of drive and desire pursued to those that are socially approved.

tally argument Theoretical justification of psychoanalysis attributed to Freud by Adolf Grünbaum, based upon the claim that correct insight is necessary for the cure of neuroses.

transference Redirection of patient feelings for parents to the therapist in the course of psychoanalysis.

wish fulfillment Symbolic expression of repressed memories and desires.

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Social and Developmental Psychology

THE RECOGNITION OF THE **SOCIAL ORIENTATION** OF HUMAN psychology and behavior—the orientation of some aspects of human thought, emotion, and behavior to the represented thought, emotion, and behavior of members of social groups—goes back to antiquity. Aristotle characterized the human being as a “social animal” (*Politics*, I, 2, 1253a2), and medieval scholars acknowledged the critical role of social community in the shaping of human psychology and behavior. Indeed in medieval times, individuals were primarily conceived of as members of social communities:

To describe an individual was to give an example of the group of which he was a member, and so to offer a particular description of that group and of the relationships within it.

—(Williams, 1961, p. 91)

Many social theorists from Plato to Émile Durkheim (1858–1917) were **holists** who conceived of social groups as supra-individual entities with emergent properties such as social minds: social forms of mentality not reducible to the mental properties of the individual persons who compose social groups. This notion was challenged by individualist social theorists in the 17th and 18th centuries, notably by Thomas Hobbes (1651/1966), who maintained that social groups are nothing more than collections of individuals and that social thought, emotion, and behavior are nothing more than the aggregate thought, emotion, and behavior of collections of individuals, governed by the pursuit of pleasure and avoidance of pain. In a similar vein, the Scottish economist Adam Smith (1723–1790) famously maintained that the distribution of social goods is a product of the individual pursuit of self-interest (Smith, 1776).

Following the Renaissance and Reformation, the medieval conception of the individual as an inseparable member of social groups was displaced by a conception of the individual as independent of social community, as an autonomous